



Draft National Policy on Health and Wellbeing Sri Lanka (2026 -2035)

Submission by National Commission on Women

20 April 2026

Introduction

The National Commission on Women is an independent Commission established under the Women Empowerment Act No. 37 of 2024. As an independent, statutory body, the Commission is mandated to promote and protect the rights of all women in Sri Lanka.

Article 2(b) of the Women Empowerment Act mandates the Commission to “introduce mechanisms to give effect to the obligations undertaken by the Government of Sri Lanka in relation to women in terms of the Convention on the Elimination of All Forms of Discrimination Against Women.” It is in line with this mandate that the Commission has reviewed the Draft National Policy on Health and Wellbeing Sri Lanka (2026-2035) and hereby submits a set of six (06) Recommendations to further strengthen and ensure the State’s obligations to CEDAW in relation to health and wellbeing.

The following national and international frameworks and policies were considered in preparing this submission:

- UN Convention on the Elimination of All Forms of Discrimination Against Women (CEDAW) [available [online](#)].
- CEDAW General Recommendation No. 24 on Article 12 of CEDAW on ‘Women and Health’ [available [online](#)].
- CEDAW Committee’s Concluding Observations on the 9th Periodic Report of Sri Lanka, 28 February 2025 [available [online](#)].
- Women Empowerment Act No. 37 of 2024 [available [online](#)]
- National Policy Framework (2024) [available [online](#)].

Recommendation No. 1

➤ Page 4, Para 4, Line 1

Existing Text:

- “Vulnerable groups, including older persons, persons with disabilities, estate communities, and the urban poor, continue to face persistent inequities in access and affordability.”

Recommended Amendment (in blue):

- Amend the existing text as follows:
- “Vulnerable groups, *including but not limited to women and girls who are victims of gender-based violence, older persons, persons with disabilities, estate communities, gender-diverse persons* and the urban poor, continue to face persistent inequities in access and affordability.”

Rationale & Justification

- Article 31(a) of CEDAW General Recommendation No. 24 on ‘Women and Health’¹ notes that States Parties should “place a gender perspective at the centre of all policies and programmes affecting women’s health.”

Recommendation No. 2

➤ Page 5, Last Para, Second Line

Existing Text:

- “The changing health landscape, with its associated ethical challenges, calls for strengthened legal and regulatory frameworks. Key areas include reproductive health, governance of municipal health services, and the ethical use of artificial intelligence in healthcare. Legal reforms are also needed to enhance private sector oversight and regulate harmful substances and products, including food, to safeguard public health.”

Recommended Amendment (in blue):

- “The changing health landscape, with its associated ethical challenges, calls for strengthened legal and regulatory frameworks. Key areas include *sexual and reproductive health and rights*, governance of municipal health services, and the ethical use of artificial intelligence in healthcare. Legal reforms are *urgently required to ensure equal access to non-discriminatory healthcare for all citizens, including vulnerable groups*. Further regulations are required to enhance private sector oversight and regulate harmful substances and products, including food, to safeguard public health.”

Rationale & Justification:

- ‘Sexual and Reproductive Health & Rights’ (SRHR) is language that has been globally recognized and adopted by the UN CEDAW Convention. Article 18 of CEDAW General Recommendation No. 24 on ‘Women & Health’ notes the following:

¹ UN Convention on Elimination of All Forms of Discrimination Against Women (CEDAW) General Recommendation No. 24 on ‘Women and Health’ [available [online](#)].

“The issues of HIV/AIDS and other sexually transmitted diseases are central to the **rights of women and adolescent girls to sexual health**. Adolescent girls and women in many countries lack adequate access to information and services necessary to ensure **sexual health**. As a consequence of unequal power relations based on gender, women and adolescent girls are often unable to refuse sex or insist on safe and responsible sex practices. Harmful traditional practices, such as female genital mutilation, polygamy, as well as marital rape, may also expose girls and women to the risk of contracting HIV/AIDS and other sexually transmitted diseases. ... In particular, States parties should ensure the rights of female and male adolescents to **sexual and reproductive health** education by properly trained personnel in specially designed programmes that respect their right to privacy and confidentiality.”

- Sri Lanka’s National Policy Framework (Page 9) refers to the following Policy Principle: ‘Health services that meet global standards’, such as the UN CEDAW Convention ratified by Sri Lanka in 1981.
- Sri Lanka’s National Policy Framework (Page 47) refers to “**comprehensive sexual and reproductive health education and services** tailored to all age groups” under ‘Key Activities’.

Recommendation No. 3

➤ Page 6, Purpose and Context

Existing Text

- *“It is also consistent with global and regional commitments, including the Sustainable Development Goals (SDGs), particularly Goal 3, the WHO Global Health and Wellbeing Framework (2025), and the Colombo Declaration on “Healthy Ageing through Strengthened Primary Health Care” (2025).”*

Recommended Amendment (in blue):

- *“It is also consistent with global and regional commitments, including **the UN Convention on the Elimination of All Forms of Discrimination Against Women (CEDAW), particularly Article 12 and General Recommendation No. 24**, the Sustainable Development Goals (SDGs), particularly Goal 3, the WHO Global Health and Wellbeing Framework (2025), and the Colombo Declaration on “Healthy Ageing through Strengthened Primary Health Care” (2025).”*

Rationale & Justification

- Sri Lanka’s National Policy Framework (Page 46) refers to ensuring ‘Domestic laws with the standards set by the UN CEDAW Convention’ as a Policy Principle.
- Sri Lanka is a State Party to the UN CEDAW Convention, having ratified the treaty in 1981, and most recently presented its 9th Periodic Report to the CEDAW Committee in February 2025.

Recommendation No. 4

➤ Page 9, Policy Statement No. 12
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Existing Text:

- “Ensure that systems for integrated surveillance, research, and citizen voice, including the participation of women and youth, are institutionalised to guide evidence-informed, responsive decision-making and build public trust.”

Recommended Amendment (in blue):

- “Ensure that systems for integrated surveillance, research, and citizen voice, including the participation of women, youth *and all other vulnerable and marginalized groups*, are institutionalised to guide evidence-informed, responsive decision-making and build public trust.”

Recommendation No. 5

➤ Page 10, Strategic Directions
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Existing Text:

- The Draft Policy does not recognize the specific needs of women and girls in relation to their right to health as determined by the UN CEDAW Convention.

Recommended Amendment (in blue):

- Suggest including a NEW Strategic Direction which will fill this critical gap.
- NEW Strategic Direction will read as follows:
- “Ensure that the health and well-being needs of women and girls are recognized and addressed, using a rights-based, life-course approach that promotes equality and equity and responds to their evolving health requirements from adolescence through older age.”

Rationale & Justification

- 51.7% of population of Sri Lanka are women² and women have specific sexual and reproductive health needs throughout their life course, from menarche to menopause and beyond.
- Article 12 of CEDAW General Recommendation No. 24 on ‘Women & Health’ notes the following:

“States parties should report on their understanding of how policies and measures on health care address the health rights of women from the perspective of women’s needs and interests and how it addresses distinctive features and factors that differ for women in comparison to men, such as:

- a) Biological factors that differ for women in comparison with men, such as their menstrual cycle, their reproductive function and menopause.

² Census of Population and Housing Final Report, 2024 [Available [online](#)]

Another example is the higher risk of exposure to sexually transmitted diseases that women face;

- b) Socio-economic factors that vary for women in general and some groups of women in particular. For example, unequal power relationships between women and men in the home and workplace may negatively affect women's nutrition and health. They may also be exposed to different forms of violence which can affect their health. Girl children and adolescent girls are often vulnerable to sexual abuse by older men and family members, placing them at risk of physical and psychological harm and unwanted and early pregnancy. Some cultural or traditional practices such as female genital mutilation also carry a high risk of death and disability.

Recommendation No. 6
➤ Page 12, Annex 1, Strategies for NEW Strategic Direction

Existing Text:

- Does not exist currently.

Recommended Amendment (in blue):

- Including the following **Strategies** under the **NEW Strategic Direction** noted in Recommendation No. 6 above: *"Ensure that the health and well-being needs of women and girls are recognized and addressed, using a rights-based, life-course approach that promotes equality and equity and responds to their evolving health requirements from adolescence through older age."*
- Suggested Strategies under the NEW Strategic Direction:
 1. Ensure that all women and girls have equal access to non-discriminatory healthcare with equity, dignity and respect to autonomy, confidentiality and informed choice, particularly on sexual and reproductive health and rights, focusing a life course approach from girlchild, adolescent, middle age to elderly.
 2. Ensure adolescent friendly health services, which promote comprehensive education on sexual and reproductive health and rights, and protects children from harmful traditional practices such as female genital mutilation, child marriage and teenage pregnancies.
 3. Ensure that all women, including vulnerable and marginalized groups, have equal access to modern family planning services and a safe pregnancy with standard antenatal and postnatal care.
 4. Strengthen mental health services for women who are pregnant or in postpartum period, victims of sexual and gender-based violence, and for women who are disproportionately burdened by unpaid care work.

5. Strengthen service delivery on preventive services including screening for NCDs, breast cancer, cervical cancer and osteoporosis.
6. Ensure health services for victims of sexual and gender-based violence through a multisectoral and coordinated approach (health, justice and social services) in delivery of survivor-centered care for victims of all forms of violence, including domestic violence.
7. Recognize the unpaid care work of women and ensure gender-sensitive health services with adequate numbers of health care professionals.
8. Encourage gender-responsive budgeting, research on women specific health issues, inclusion of women in clinical trials, gender disaggregated data management systems to ensure measurable progress, toward equitable and gender sensitive health outcomes.

Rationale and Justification

- Sri Lanka's National Policy Framework (Page 47) refers to “comprehensive sexual and reproductive health education and services tailored to all age groups” under ‘Key Activities’.
- The following data also demonstrates why it is important to recognize and include the Recommended Strategies as mentioned above:
 - 3.8% of pregnant mothers in Sri Lanka³ are teenage girls who are highly vulnerable to obstetric complications.
 - Approximately 15% of adolescents and young adults between 15 – 24 years of age are sexually active.⁴
 - Breast Cancer, which is the most common cancer⁵, is diagnosed late in Sri Lanka.
 - Women are more vulnerable to Non-Communicable Diseases as evidenced by the fact that 44% of women are either overweight or obese and 14.7 of women are diabetics.⁶

³ Issue Brief, Teenage Pregnancy in Sri Lanka: Trends and Determinants, UNFPA, 2024 [available [online](#)]

⁴ National Youth Health Survey Sri Lanka (2012/2013), Family Health Bureau, 2015 [available [online](#)]

⁵ Data from National Cancer Control Programme.

⁶ Non-Communicable Diseases Risk Factor Survey (STEPS Survey), Ministry of Health and Department of Census and Statistics, Sri Lanka, 2021 [Available [online](#)].